Global Advocacy Data Hub (GADH) statement to the 56th Global Fund Board

15 October 2026

The Global Advocacy Data Hub (GADH) is a civil society-led platform and network that builds the power of communities in all their diversities by providing the information, data, resources, and support they need to engage in national and global health decision-making and hold health systems and donors accountable. The GADH aims to center and empower communities, particularly those most affected by health disparities and often excluded from decision-making spaces, by promoting transparency, facilitating access to data, generating community evidence, and promoting peer support.

The Global Advocacy Data Hub (GADH) recognizes the exceptionally challenging context facing the Global Fund. While some donors have increased their contributions, reduced and unconverted pledges from several of the Global Fund's largest donors have created an unprecedented level of disruption to the entire partnership. We acknowledge the **tremendous efforts made by the Secretariat and Board to mitigate the harm** of this rapid funding withdrawal.

It is still unknown whether the Global Fund will succeed in protecting the life-saving systems and infrastructure it has helped build. The timing of the 56th Board Meeting invites such reflection. By the end of 2026, the Technical Review Panel will have reviewed 86% of the Global Fund's portfolio, and grant implementation for many countries will begin in January 2027. **We call on the Board to reflect on early signals emerging from 2026**, while there is still time to address emerging gaps.

The GADH acknowledges the major tensions facing the Secretariat and Board as it balances its fiduciary responsibilities, its strategic objectives, and the evolving global health architecture. We also see the emerging global health architecture as an opportunity to strategically strengthen health systems and country ownership. At this Board meeting, we urge the Board to recall that **country ownership means ownership not only by government, but also by communities, civil society organizations (CSOs),** and healthcare service users, private sector, the healthcare workforce, faith-based organizations, academia, and more. Protecting and centering the communities living with and most affected by the three diseases aligns with, and enables, the Accra Reset and the Lusaka Agenda.

The Global Advocacy Data Hub (GADH) offers several recommendations to inform Board discussions, based on early findings from the Msingi Study. The Msingi Study is an independent, civil society-led, mixed-methods research study led by the Global Advocacy Data Hub, which focuses on documenting in-country experiences planning for program integration and Global Fund transition. To date, 379 CCM representatives and community partners engaged in Global Fund consultations have participated, with data from 76 countries.

1. Ensure that Global Fund reporting frameworks are fit for purpose.

The Global Fund's 2023-2028 Strategy was developed several years before the Eighth Replenishment and the launch of the GC8 Shifts. The Board must therefore confirm that **its monitoring mechanisms adequately capture the impact of the GC8 Shifts**, particularly in relation to program integration and transition.

The rapid pace of integration and transition is **driven by financial uncertainty, and not country readiness**, as historically understood in the Sustainability, Transition and Co-Financing (STC) Policy. As such, the existing KPI Framework must be adapted to capture the new and evolving risks associated with the GC8 Shifts and the current challenging and rapidly-changing social, economic and political environment.

We request the Global Fund to:

1. **Enhance monitoring of transition and integration.** The Global Fund Board must mandate adequate monitoring, evaluation, and reporting on country transition and integration. This should include routine reporting, such as through KPIs, with transparent reporting to the Board and partnership.
2. **Develop a light-touch reporting mechanism for countries beyond transition**, in partnership with its technical partners and with the meaningful participation of communities and CSOs, with a focus on epidemiological rebound, community systems, and the continuity of Global Fund's programmatic investments.
3. **Commission assessments of post-transition countries.** The Office of the Inspector General (OIG) or other independent evaluation body should commission and publish a retrospective analysis of countries that have transitioned in the past 2-3 cycles, including transitions that have ultimately failed to preserve HIV, tuberculosis, and malaria programs. This analysis must demonstrate successful and replicable approaches, as well as clear lessons learned about how transitions can fail.
4. **Develop partnerships to safeguard oversight and support post-transition.** As the Secretariat defines its strategies for engaging in the new global health architecture, prioritize new mechanisms and partnerships with technical partners and other international organizations that will have a presence beyond Global Fund transition. These partnerships should facilitate support for civil society accountability activities and facilitate monitoring of transition outcomes.

2. Commit to a minimum principle of 'do no harm' in integration and transition planning.

Findings from the Msingi Study clearly show that in most countries, serious barriers to care in primary healthcare (PHC) settings remain unaddressed. These include overburdened human resources, fragile supply chains, and significant disruption due to the termination of U.S. government contracts. In such contexts, shifting service delivery into PHC risks creating new barriers to care for Global Fund-supported clients and the broader healthcare system.

Additionally, more serious challenges are frequently reported, including increasingly aggressive enforcement of criminalization, the rise of the anti-gender movement and far-right governments,

political violence, and active military conflict. In such contexts, requiring **transition and integration into PHC is a human rights issue**. It is incumbent upon the Global Fund to maintain a very high level of scrutiny and situational awareness in the implementation of the GC8 Shifts.

We request the Global Fund to:

5. **Require technical assessments of transition and integration barriers during grant-making.** For the remainder of Funding Requests reviewed in GC8, and looking ahead to GC9, provide the Technical Review Panel (TRP) and Grant Approvals Committee (GAC) with sufficient analysis of the legal and regulatory barriers to integration, transition, and social contracting to ensure that grants are designed to address these gaps. This is aligned with the TRP's own observations that 30% of Funding Requests submitted in Windows 1 and 2 did not identify sustainability and transition challenges and provide clear plans, actions, and investments to address them. Assessments of transition readiness provided in Funding Requests must be complemented by independent expert assessment, in partnership with technical partners.
6. **Protect dual-track financing until all barriers have been reasonably addressed.** We urge that the streamlining of Principal Recipients (PRs) be paused until a full legal and policy assessment has demonstrated the capacity, willingness, and appropriateness of the government PR – or another funder – to assume the programs implemented under the non-governmental PR(s). This should at minimum include a demonstrated lack of criminal penalties or risk of arrest to communities accessing care in PHC settings, and demonstration of a social contracting mechanism or other mechanism for community-led organizations to deliver peer-led services. In the absence of such documentation, dual-track financing must be preserved until the country context has sufficiently evolved.

3. Scale up dissemination of guidance and information related to integration and transition.

Msingi Study findings reveal the **lack of a shared understanding of integration** in many contexts. Whereas most stakeholders understand integration to mean the delivery of services at a single point of care, in many contexts there is growing pressure to shift all services into primary healthcare settings, at the expense of community- and CSO-based services. In the absence of clear definitions and directives about minimum standards, this ambiguity can be exploited in ways that undermine community-focused care.

We request the Global Fund to:

7. **Publish clear and directive guidance around integration.** Re-publish the Integration Technical Brief on the Global Fund's website. Removing this document during Funding Request development is a barrier to access to basic information and guidelines around integration. Enhance this guidance with specific and directive requirements about basic

minimum criteria for successful integration, in line with the Global Fund's Strategy and commitment to access to care and protecting human rights.

8. **Increase information sharing to civil society.** Working with the Regional Learning Hubs, enhance outreach and information-sharing around integration and transition to ensure that all stakeholders understand the concepts, timelines, and guidelines needed to meaningfully engage in decision-making.
9. **Publish transition plans.** To allow shared ownership and oversight, transition plans must be publicly-available and include measurable milestones toward transition readiness.

4. Leverage standard reporting requirements to facilitate visibility into global health investments.

The findings from the Msingi Study identify transparency as a challenge facing the CCM during Funding Request development and grant-making. Notably, the concurrent negotiation of bilateral funding agreements between implementing governments and the U.S. government limited the CCM's visibility into the pool of resources available for health programs in countries. In parallel, the recent termination of key U.S. government-funded global health awards created additional confusion about the funding landscape. While recognizing that bilateral agreements are not Global Fund-owned processes, we urge the Secretariat to use its existing reporting requirements to promote transparency and information-sharing.

10. **Facilitate visibility into bilateral investments into health.** The Global Fund must take all efforts possible to promote the transparency of bilateral government contributions toward HIV, tuberculosis, and malaria. At minimum, this must include requiring that modular-level data about sources of funds be collected (as part of the Funding Request's Funding Landscape Table) and included in the published copy of the document on the Global Fund's data portal.

5. Mobilize the Secretariat's full capacity to ensure effective transitions.

Findings from the Msingi Study, including findings from post-transition countries, suggest an **instrumental role for the Global Fund Secretariat in achieving successful transitions**. Study participants describe an urgent need for high-level engagement with governments in transition planning and increased investments into transition and integration planning. We are concerned by findings that CCMs in some contexts have in practice ceased to function, due to the exit of UNAIDS, and the shift in political dynamics as longtime community allies on the CCMs withdraw.

In this pivotal period, we see a key role for Global Fund's voice that draws on the Secretariat's full range of tools and influence, while simultaneously protecting the core principle of country ownership.

We request the Global Fund to:

11. **Actively verify that CCMs are well-functioning.** The Secretariat must confirm that disruption to UNAIDS and closures of country offices are not affecting CCM functioning, staffing, and facilitation. This must include heightened scrutiny of all minimum requirements for CCM functioning by the Compliance Review Panel (CRP), including but not limited to CCM Eligibility Requirement #1, according to the CCM Policy and other Global Fund guidelines.
12. **Increase high-level engagement with governments.** In this period of major evolution to the Global Fund partnership, the Global Fund Secretariat must increase its high-level engagement with governments around transition. This engagement must promote not only political declarations, but legally-binding instruments documenting governments' commitments to preserve the Global Fund's investments beyond transition and during integration. The Global Fund must ensure that the commitments it solicits from governments are not administration- or individual-specific, but rather can survive transitions of power, loss of institutional memory, and shifting country political and epidemiological contexts.
13. **Protect community voice on CCMs.** Beyond its high-level engagement, the Secretariat must work in partnership with its technical partners to protect community and CSO voices in CCMs and, where possible, other national accountability spaces. This is an urgent priority as institutions that previously acted as advocates for human rights and community-focused activities (e.g. the U.S. government and UNAIDS) have shifted their priorities or have reduced capacity to adequately engage. Additionally, the CCM Hub and OIG must increase monitoring of retaliation and risks to safety, particularly in criminalized contexts.
14. **Mobilize resources to scale up integration- and transition-focused Catalytic Investments.** Pursue aggressive efforts to mobilize additional public and private resources for Board approved transition- and integration-focused Catalytic Investment priorities, including "Supporting Sustainability and Transition from Global Fund Financing", "Addressing Human Rights & Gender Barriers to Services", and "Building Community Networks & Engagement to Enable Access to Services." Funding levels must be high enough to provide multi-year support across the entire portfolio. These resources should be used for urgently addressing risks to community-focused programs (including human rights and gender-related barriers) during integration and beyond transition, community-led advocacy, civil society-led monitoring and evaluation of the primary healthcare (PHC) system, and the training and upskilling needed to make integrated clinics safe and accessible.

6. Urgently address the elevated risk of civil society collapse post-transition.

Data from the Msingi Study make clear that **Global Fund resources are the last remaining source of funding for community-led organizations and CSOs** delivering healthcare services, advocating for accountable healthcare systems, and providing the peer support needed to ensure that all people living with and affected by the three diseases receive healthcare. Social contracting is one pathway to reimburse community-led services and sustain

civil society. However, Msingi data indicate that social contracting mechanisms are not functional in most countries, and few alternative funding pathways for community organizations were identified. The evidence of dissolution of civil society organizations in post-transition countries highlights this acute risk.

We further note that in many countries, the creation of the **CCM has opened up engagement and accountability opportunities not previously available** to communities. However, evidence from Msingi suggests that functional CCMs typically do not persist beyond transition. While recognizing that the CCMs were designed for the Global Fund's grant-making model, we urge the Global Fund to recognize that, in post-transition contexts, the absence of democratic accountability spaces risks undermining more than two decades of investment in health systems. Msingi data additionally highlight a concerning lack of visibility into disaggregated government data on budgets, expenditures, and programmatic outcomes, further undermining government oversight.

We request the Global Fund to:

15. **Invest in government transparency.** Global Fund investments into data systems and digital health must include investments in basic, public transparency of government budgets, expenditures, and programmatic outcomes. This should include investments made through the GC8 grants, as well as the Digital Health Impact Accelerator Strategic Initiative and the Matching Funds for Public Financial Management and Digital Solutions Supporting Integration.
16. **Create multicountry funding mechanisms to support accountability.** In GC9, use catalytic investments to support a multi-country funding modality for civil society advocacy, oversight, and accountability activities in transitioning and post-transition countries. We suggest these could be invested through a multicountry mechanism, to reduce administrative burden to the Secretariat, and should include sufficient resources to match the CCM's own budgets for community engagement (i.e. 15% of the CCM Funding Agreement, per Global Fund's Operational Policy Manual) for at minimum one Allocation Cycle post-transition. These resources should be accompanied by technical assistance focused on legal barriers to government ownership, domestic budget monitoring, and other targeted transition-related topics.
17. **Require demonstrations at scale of social contracting.** Use Catalytic Investments to support the development and implementation of legal and policy frameworks that enable social contracting and sustain community-led and CSOs service delivery beyond small-scale projects.

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